Inguinoscrotal Examination — OSCE Checklist

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional, verbalised or done if indicated.* † *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **Introduction → Standing: inspect, palpate, special tests → Supine: re-examine, reduce, transilluminate, percuss, auscultate**. *Not assessed practically in Year 2, but assessable in written form.*

1. Introduction (intimate examination)

☐	**Introduce** self (name, role); confirm patient ID
☐	**Explain** what the exam involves and why; check understanding
☐	**Offer a chaperone / observer** (document if declined)
☐	**Consent**: verbal, informed; "Just say stop and I will"
☐	**Wash hands**; gloves where indicated
☐	**Privacy**: undress behind curtain, drape/cover, warm room, good light
☐	Sheet provided; **gloves on both hands** for the whole examination
☐	Sign outside the cubicle so you are not interrupted
☐	Start with patient **standing**; undress lower garments behind curtain (gown if preferred)
☐	Ask patient to show you the area of concern; ensure adequate exposure (umbilicus to mid-thigh)

2. Inspection — standing

	Inspect	Look for
☐	Skin of lower abdomen, inguinal regions, upper inner thighs, scrotum	Scars, erythema, rashes
☐	Swellings / lumps	Groin or scrotal; note side
☐	Testicular asymmetry	Left usually hangs lower than right (normal)
☐	Scrotal oedema	Note extent
☐	Cough impulse — visible	"Please turn your head and cough": look for bulge each side. No impulse in obstructed, strangulated or omental hernia

3. Palpation — inguinal region (standing)

☐	Compare both sides: palpate from pubic tubercle to ASIS, above and below the inguinal ligament
☐	Inguinal lymph nodes; femoral artery pulse
☐	Any swelling: warmth, tenderness, consistency
☐	Extent: can you reach its top and bottom? Does it descend into the scrotum?
☐	Relation to **pubic tubercle** and inguinal ligament: above-medial = inguinal; below-lateral = femoral †

4. Palpation — scrotum (standing)

☐	Warn patient; ask them to say if there is any pain
☐	Palpate one testis at a time between thumb and fingertips (or cradle with middle + ring fingers, thumb above)
☐	Both testes present? Estimate size of each; compare
☐	Palpate testis, epididymis (posterior) and spermatic cord
☐	Warmth, tenderness, swellings or lumps; consistency
☐	Can you **get above** the swelling? Gently pinch the upper scrotum between thumb and fingers

5. Special tests (standing)

	Test	How	Interpretation
☐	Cough impulse — palpated	Fingers over pubic tubercle region; patient coughs; each side	Expansile impulse = hernia
☐	Deep ring occlusion	Reduce, then press over deep ring (midpoint of inguinal ligament †); cough; each side	Controlled = indirect; still protrudes = direct
☐	Cremasteric reflex*	Stroke upper inner thigh	Testis rises; lost in torsion

6. Supine

	Step	How	Interpretation
☐	Re-examine	Re-palpate any swelling found standing	Exact site and nature clearer supine
☐	Reducibility	Attempt gentle manual reduction; feel for gurgling	Irreducible = incarcerated; tender + irreducible = strangulation
☐	Transillumination	Darkened room, torch against scrotal swelling	Lights up = cystic; dark = solid
☐	Percussion	Percuss over swelling	Hollow = gas-filled bowel; splash = obstructed loop
☐	Auscultation	Over the mass and the abdomen	Bowel sounds = bowel in hernia

7. Closing

☐	Check for undue pain or distress
☐	Privacy to dress
☐	Explain findings and next steps; invite questions
☐	Restore examination area/couch
☐	Document consent, chaperone, examination, findings and discussion
☐	Completion statement: "I would also examine the abdomen and external genitalia †"

8. Quick reference

Hernia †	Site	Deep ring occlusion	Into scrotum?	Other
Indirect inguinal	Through deep ring, lateral to inferior epigastric vessels	Controlled	Often	Most common; younger men
Direct inguinal	Hesselbach's triangle, medial to inferior epigastric vessels	Not controlled	Rarely	Older; weak posterior wall
Femoral	Below and lateral to pubic tubercle	—	No	More common in women; high strangulation risk

Scrotal lump †	Get above?	Separate from testis?	Transilluminates?	Clues
Inguinoscrotal hernia	No	Yes	No	Cough impulse, may reduce
Hydrocele	Yes	No — testis within	Yes	Smooth, painless
Epididymal cyst	Yes	Yes (above / behind)	Yes	Small, smooth
Varicocele	Yes	Yes	No	"Bag of worms", left, empties lying down
Testicular tumour	Yes	No — part of testis	No	Hard, painless, irregular; urgent US
Torsion / epididymo-orchitis	Yes	—	No	Torsion: acute pain, high-riding, absent cremasteric — surgical emergency